

Started on Thursday, 11 May 2023, 3:22 PM
State Finished
Completed on Thursday, 11 May 2023, 3:22 PM
Time taken 8 secs
Grade 0.00 out of 10.00 (0%)

Question 1

Not answered

Marked out of 1.00

A 55 years old male with history of HTN, DM presents with "crushing" substernal Chest Pain, diaphoresis, hypotension, tachycardia and cool, clammy extremities.

Select one:

- ☐ a. Hypovolemic shock
- ☐ b. Anaphylactic shock
- ☐ c. Obstructive shock
- ☐ d. Cardiogenic shock

The correct answer is: Cardiogenic shock

Question 2

Not answered

Marked out of 1.00

40 years old female patient that developed fever and was diagnosed to have COVID-19 infection, during the course of treatment she developed tachypalpitations and with central chest pain, on examination, pulse+AD0-150 b/m and hypotensive, ECG revealed sinus tachycardia, what is the next important step in diagnosis:

Select one:

- ☐ a. CT chest
- ☐ b. ABG
- ☐ c. Echocardiography
- ☐ d. D-Dimer

The correct answer is: Echocardiography

Question 3

Not answered

Marked out of 1.00

An 81 years old female resident of a nursing home presents to the ED with altered mental status. She is febrile to 39.4, hypotensive with a widened pulse pressure, tachycardic, with warm extremities.

Select one:

- ☐ a. Anaphylactic shock
- ☐ b. Cardiogenic shock
- ☐ c. Obstructive shock
- ☐ d. Septic shock

The correct answer is: Septic shock

Question 4

Not answered

Marked out of 1.00

66 years old male patient presented to ER by severe compressing chest pain, he is cold and hypotensive, ECG revealed acute extensive anterior myocardial infarction, what is next important step in management:

Select one:

- ☐ a. Vasopressor infusion.
- ☐ b. Troponin I level
- ☐ c. Refer to Cath lab for primary PCI
- ☐ d. Streptokinase infusion

The correct answer is: Refer to Cath lab for primary PCI

Question 5

Not answered

Marked out of 1.00

A 34 years old Female presents to the ER after dining at a restaurant where shortly after eating the first few bites of her meal, became anxious, diaphoretic, began wheezing, noted diffuse pruritic rash, nausea, and a sensation of her "throat closing off". She is currently hypotensive, tachycardic and ill appearing.

Select one:

- ☐ a. Cardiogenic shock
- ☐ b. Hypovolemic shock
- ☐ c. Anaphylactic shock
- ☐ d. Obstructive shock

The correct answer is: Anaphylactic shock

Question 6

Not answered

Marked out of 1.00

A 41 years old male presents to the ER after an Motor Vehicle Crush (MVC) complaining of decreased sensation below his waist and is now hypotensive, bradycardic, with warm extremities.

Select one:

- ☐ a. Neurogenic shock
- ☐ b. Cardiogenic shock
- ☐ c. Anaphylactic shock
- ☐ d. Hypovolemic shock

The correct answer is: Neurogenic shock

Question 7

Not answered

Marked out of 1.00

65 years old female patient was admitted to hospital for an orthopedic surgery, reduction and internal fixation of fracture neck femur , at the day of discharge when her relatives try to assist her to get off the bed, she became collapsed, cold, tachycardic.

Select one:

- ☐ a. Cardiogenic shock
- ☐ b. Hypovolemic shock
- ☐ c. Obstructive shock
- ☐ d. Anaphylactic shock

The correct answer is: Obstructive shock

Question 8

Not answered

Marked out of 1.00

68 years old male with history of HTN and DM presents to the ER with abrupt onset of diffuse abdominal pain with radiation to his low back. The patient is hypotensive, tachycardic, afebrile, with cool but dry skin.

Select one:

- ☐ a. Obstructive shock
- ☐ b. Cardiogenic shock
- ☐ c. Hypovolemic shock
- ☐ d. Anaphylactic shock

The correct answer is: Hypovolemic shock

Question 9

Not answered

Marked out of 1.00

A 37 year-old otherwise healthy woman comes in to your ED complaining of an itchy red rash that started soon after eating some seafood at a restaurant. She complains of a scratchy feeling in her throat, but denies any difficulty breathing, speaking or swallowing. She denies lip or tongue swelling, SOB, abdominal pain, vomiting or fever. On exam, her vitals are normal. She appears uncomfortable, scratching at her neck with an obvious raised patchy red urticarial rash all over her neck and torso. Her voice is normal, lips and tongue do not appear swollen, theres no stridor and her chest is clear.

Select one:

- ☐ a. Anaphylactic reaction
- ☐ b. Food poisoning
- ☐ c. Foreign body inhalation
- ☐ d. Anaphylactic shock

The correct answer is: Anaphylactic reaction

Question 10

Not answered

Marked out of 1.00

A 24 years old male presents to the ED after an MVC c/o chest pain and difficulty breathing. On examination , you note the patient to be tachycardic, hypotensive, hypoxic, and with decreased breath sounds on left.

Select one:

- ☐ a. Hypovolemic shock
- ☐ b. Cardiogenic shock
- ☐ c. Anaphylactic shock
- ☐ d. Obstructive shock

The correct answer is: Obstructive shock

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Time taken 13 secs
Marks 0.00/11.00
Grade **0.00** out of 10.00 (0%)

Question 1

Not answered

Marked out of 1.00

Which test is not useful in a patient with history of syncopal attack

Select one:

- ☐ a. CT chest
- ☐ b. Electrophysiological test
- ☐ c. Tilt table test
- ☐ d. Holter monitoring

The correct answer is: CT chest

Question 2

Not answered

Marked out of 1.00

35 years old healthy male presents with an episode of syncope while working out at the gym and physical exam showed a grade III systolic murmur that decreased with Valsalva with palpable thrill. The expected cause of syncope is

Select one:

- ☐ a. AS
- ☐ b. MS
- ☐ c. postural hypotension
- ☐ d. heart block

The correct answer is: AS

Question 3

Not answered

Marked out of 1.00

A patient who has experienced a vasovagal syncopal episode will most often:

Select one:

- ☐ a. experience a postictal-like period afterward.
- ☐ b. remain unconscious for a long period of time.
- ☐ c. quickly regain consciousness after being positioned supine.
- ☐ d. present with tachycardia and hypertension.

The correct answer is: quickly regain consciousness after being positioned supine.

Question 4

Not answered

Marked out of 1.00

A 30-year-old female experienced a sudden loss of consciousness last approximately 5 minutes. Blood pressure as low as 80 mm Hg. History revealed that she had experienced several episodes presyncope and loss of consciousness. A feeling of nausea and light-headedness would always precede the attack. She had never been observed to have a typical seizure. On recovery, she would feel systemically unwell, light-headed, and nauseated for several hours. Tilt-table test was advised. what is the cause of patient symptom?

Select one:

- ☐ a. cerebrovascular stroke
- ☐ b. head trauma
- ☐ c. vasovagal syncope
- ☐ d. cardiogenic syncope

The correct answer is: vasovagal syncope

Question 5

Not answered

Marked out of 1.00

A 50-year-old woman presented to the emergency department after an episode of syncope while at work. She had one episode of syncope several years earlier. There was associated chest pain, no diaphoresis, or sensation of warmth, but she experienced palpitations before the syncope. She had no postictal state her systolic blood pressure was found to be 80mm Hg. Which of the following is the most likely etiology of syncope in this patient?

Select one:

- ☐ a. Cardiac arrhythmia
- ☐ b. Vasovagal syncope
- ☐ c. Pulmonary embolism
- ☐ d. Valvular heart disease
- ☐ e. Recreational drug use

The correct answer is: Cardiac arrhythmia

Question 6

Not answered

Marked out of 1.00

An 80 year-old man presents with episodes of unexplained presyncope. He is taking a diuretic and beta blocker for hypertension. Examination discloses a heart rate of 55 per minute with a regular rhythm. Blood pressure is 110/70. The most likely cause of his attacks, which occur mainly in the morning, is:

Select one:

- ☐ a. Postural hypotension
- ☐ b. Heart block
- ☐ c. Neuro-cardiogenic syncope
- ☐ d. Chronotropic incompetence with sinus bradycardia
- ☐ e. Ventricular arrhythmias

The correct answer is: Postural hypotension

Question 7

Not answered

Marked out of 1.00

35 years old healthy male presents with an episode of syncope while working out at the gym and physical exam showed a grade III systolic murmur that decreased with Valsalva with palpable thrill. The next step to confirm the diagnosis is:

Select one:

- ☐ a. ECG
- ☐ b. Holter monitoring
- ☐ c. cardiac biomarkers
- ☐ d. Echocardiography

The correct answer is: Echocardiography

Question 8

Not answered

Marked out of 1.00

A syncopal episode is most likely to occur when the patient is in which position?

Select one:

- ☐ a. standing
- ☐ b. kneeling
- ☐ c. supine
- ☐ d. sitting

The correct answer is: standing

Question 9

Not answered

Marked out of 1.00

Which of the following is a disorder that can cause Palpitations?

Select one:

- ☐ a. Mesothelioma
- ☐ b. Bronchogenic carcinoma
- ☐ c. An adrenal gland tumor
- ☐ d. Prostate cancer

The correct answer is: An adrenal gland tumor

Question 10

Not answered

Marked out of 1.00

A 22-year-old male with no significant past medical history is brought to the emergency department for syncope. He was getting a haircut when he fainted. On hospital arrival his blood pressure was in the 60s and his heart rate was in the low 30s. The patient states that he has fainted many times in the past when getting a haircut. He denies palpitations, chest pain, pulse of 48.+AKA- Exam is otherwise normal. Initial diagnosis is:

Select one:

- ☐ a. ACS
- ☐ b. Hypothyroidism
- ☐ c. Bradydysrhythmia and CSS
- ☐ d. Vasovagal episode

The correct answer is: Bradydysrhythmia and CSS

Question 11

Not answered

Marked out of 1.00

A 65 years old male with no previous history of cardiac diseases, presented by syncope examination discloses a regular rhythm, heart rate 44/minutes with intermittent pauses the heart rate increases normally with exercises. On hospital arrival and after ECG, doctor advised placement of a dual chamber pacemaker. The most likely diagnosis is

Select one:

- ☐ a. Tachyarrhythmias
- ☐ b. Brief paroxysms of atrial fibrillation
- ☐ c. Intermittent complete heart block
- ☐ d. Intermittent sinus bradycardia
- ☐ e. Extrasystoles

The correct answer is: Intermittent complete heart block

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State Finished
Completed on Saturday, 20 May 2023, 1:28 PM
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Grade 0.00 out of 10.00 (0%)

Question 1

Not answered

Marked out of 1.00

Heavy Smoker male patient 62 years old, presented by acute retrosternal burning chest pain for 3 hours, followed by dyspnea, orthopnea, respiratory distress, hypotension, which of the following is the first investigation:

Select one:

- ☐ a. 12 leads ECG
- ☐ b. Echocardiography
- ☐ c. Cardiac troponin
- ☐ d. BNP

The correct answer is: 12 leads ECG

Question 2

Not answered

Marked out of 1.00

Male patient 68 years old, known to have ischemic cardiomyopathy, presented by severe pulmonary edema, all of the following can be a precipitating factor except:

Select one:

- ☐ a. Pulmonary embolism
- ☐ b. Aortic dissection
- ☐ c. Severe hypertension
- ☐ d. Rapid AF

The correct answer is: Pulmonary embolism

Question 3

Not answered

Marked out of 1.00

Female patient 65 years old, known to have ischemic cardiomyopathy, EF=33%, ECG:left bundle branch block, after improvement of pulmonary edema, which of the following is considered to be appropriate for HF treatment ??

Select one:

- ☐ a. Spironolactone administration
- ☐ b. ACE inhibitors administration
- ☐ c. Antiplatelets
- ☐ d. All of the above

The correct answer is: All of the above

Question 4

Not answered

Marked out of 1.00

Male patient known to have IHD, presented by dyspnea, respiratory distress, o/e, there was raised rapid irregular pulse, raised JVP, bilateral basal crepitation, which of the following is correct:

Select one:

- ☐ a. Anticoagulation
- ☐ b. IV digoxin
- ☐ c. All of the above
- ☐ d. IV furosemide

The correct answer is: All of the above

Question 5

Not answered

Marked out of 1.00

Male patient 68 years old, known to have ischemic cardiomyopathy, presented by severe pulmonary edema, with refractory hypoxia, but hemodynamically stable all of the following are correct to correct hypoxia except:

Select one:

- ☐ a. IV antibiotics
- ☐ b. CPAP
- ☐ c. Increase dose of nitrates.
- ☐ d. Further doses of IV furosemide (40-80mg)

The correct answer is: IV antibiotics

Question 6

Not answered

Marked out of 1.00

Female patient 70 years old, with history of diabetes, hypertension, presented by pulmonary edema to ED, which of the following is considered to be high risk features:

Select one:

- ☐ a. Serum Creatinine 1.2 mg/dl
- ☐ b. Serum Sodium 133 mmol/l
- ☐ c. Blood pressure 170/100 mmHg
- ☐ d. Elevated troponin I

The correct answer is: Elevated troponin I

Question 7

Not answered

Marked out of 1.00

Heavy Smoker male patient 60 years old, presented by dyspnea, orthopnea, respiratory distress, o/e: blood pressure:130/80mmHg, HR:110b/m, with raised JVP, S3 Gallop, SaO2 on room air was 95%, the most important next step:

Select one:

- ☐ a. IV propranolol
- ☐ b. IV furosemide
- ☐ c. IV verapamil
- ☐ d. Oxygen supplementation

The correct answer is: IV furosemide

Question 8

Not answered

Marked out of 1.00

Female patient 60 years old, presented by dyspnea, orthopnea, respiratory distress, o/e: blood pressure:105/75mmHg, HR:105b/m, SaO2:95%, all of the following are correct except:

Select one:

- ☐ a. IV furosemide
- ☐ b. Oxygen supplementation
- ☐ c. IV nitrates
- ☐ d. IV morphine

The correct answer is: Oxygen supplementation

Question 9

Not answered

Marked out of 1.00

Male patient 65 years old, with history of uncontrolled diabetes, hypertension, presented by pulmonary edema to ED, all of the following are signs suggestive of pulmonary edema except:

Select one:

- ☐ a. Collapse neck veins.
- ☐ b. Rapid weak pulse
- ☐ c. Bilateral basal crepitations
- ☐ d. S3 Gallop

The correct answer is: Collapse neck veins.

Question 10

Not answered

Marked out of 1.00

Female patient 70 years old, presented by dyspnea, orthopnea, respiratory distress, o/e: blood pressure:70/40mmHg, HR:125b/m, SaO2:92%, the most appropriate step is :

Select one:

- ☐ a. IV nitrates
- ☐ b. IV morphine
- ☐ c. IV frusemide
- ☐ d. IV inotropes

The correct answer is: IV inotropes

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Completed on Saturday, 20 May 2023, 1:38 PM
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Grade 0.00 out of 10.00 (0%)

Question 1

Not answered

Marked out of 1.00

Male patient 68 years old, known to have ischemic cardiomyopathy, presented by severe pulmonary edema, with refractory hypoxia, but hemodynamically stable all of the following are correct to correct hypoxia except:

Select one:

- ☐ a. IV antibiotics
- ☐ b. Increase dose of nitrates.
- ☐ c. CPAP
- ☐ d. Further doses of IV frusemide (40-80mg)

The correct answer is: IV antibiotics

Question 2

Not answered

Marked out of 1.00

Female patient 70 years old, presented by dyspnea, orthopnea, respiratory distress, o/e: blood pressure:70/40mmHg, HR:125b/m, SaO2:92+ACU-, the most appropriate step is :

Select one:

- ☐ a. IV frusemide
- ☐ b. IV nitrates
- ☐ c. IV morphine
- ☐ d. IV inotropes

The correct answer is: IV inotropes

Question 3

Not answered

Marked out of 1.00

Female patient 65 years old, known to have ischemic cardiomyopathy, EF+AD0-33+ACU-, ECG:left bundle branch block, after improvement of pulmonary edema, which of the following is considered to be appropriate for HF treatment ??

Select one:

- ☐ a. ACE inhibitors administration
- ☐ b. All of the above
- ☐ c. Biventricular pacing
- ☐ d. Spironolactone administration

The correct answer is: All of the above

Question 4

Not answered

Marked out of 1.00

Heavy Smoker male patient 62 years old, presented by acute retrosternal burning chest pain for 3 hours, followed by dyspnea, orthopnea, respiratory distress, hypotension, which of the following is the most important investigation:

Select one:

- ☐ a. Cardiac troponin
- ☐ b. Echocardiography
- ☐ c. 12 leads ECG
- ☐ d. BNP

The correct answer is: 12 leads ECG

Question 5

Not answered

Marked out of 1.00

Female patient 60 years old, presented by dyspnea, orthopnea, respiratory distress, o/e: blood pressure:105/75mmHg, HR:105b/m, SaO₂:93+ACU-, all of the following are correct except:

Select one:

- ☐ a. Oxygen supplementation
- ☐ b. IV furosemide
- ☐ c. IV nitrates
- ☐ d. IV morphine

The correct answer is: Oxygen supplementation

Question 6

Not answered

Marked out of 1.00

Male patient 65 years old, with history of uncontrolled diabetes, hypertension, presented by pulmonary edema to ED, all of the following are signs suggestive of pulmonary edema except:

Select one:

- ☐ a. Bilateral basal crepitations
- ☐ b. Rapid weak pulse
- ☐ c. S3 Gallop
- ☐ d. Collapse neck veins.

The correct answer is: Collapse neck veins.

Question 7

Not answered

Marked out of 1.00

Male patient known to have IHD, presented by dyspnea, respiratory distress, o/e, there was raised rapid irregular pulse, raised JVP, bilateral basal crepitations, which of the following is correct:

Select one:

- ☐ a. IV furosemide
- ☐ b. IV digoxin
- ☐ c. All of the above
- ☐ d. Anticoagulation

The correct answer is: All of the above

Question 8

Not answered

Marked out of 1.00

Male patient 68 years old, known to have ischemic cardiomyopathy, presented by severe pulmonary edema, all of the following can be a precipitating factor except:

Select one:

- ☐ a. Aortic dissection
- ☐ b. Severe hypertension
- ☐ c. Rapid AF
- ☐ d. Pulmonary embolism

The correct answer is: Pulmonary embolism

Question 9

Not answered

Marked out of 1.00

Female patient 70 years old, with history of diabetes, hypertension, presented by pulmonary edema to ED, which of the following is considered to be high risk features:

Select one:

- ☐ a. Serum Sodium 133 mmol/l
- ☐ b. Serum Creatinine 1.2 mg/dl
- ☐ c. Blood pressure 170/100 mmHg
- ☐ d. Elevated troponin I

The correct answer is: Elevated troponin I

Question 10

Not answered

Marked out of 1.00

Heavy Smoker male patient 60 years old, presented by dyspnea, orthopnea, respiratory distress, o/e: blood pressure:130/80mmHg, HR:110b/m, with raised JVP, S3 Gallop, SaO2 on room air was 95+ACU-, the most important next step:

Select one:

- ☐ a. IV propranolol
- ☐ b. Oxygen supplementation
- ☐ c. IV verapamil
- ☐ d. IV furosemide

The correct answer is: IV furosemide

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Completed on Saturday, 27 May 2023, 3:03 PM
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Grade 0.00 out of 10.00 (0%)

Question 1

Not answered

Marked out of 1.00

18 years old young man presented by hypertension which is 160/90 in upper limb and 120/75 in lower limbs What is the first possible cause in this case?

Select one:

- ☐ a. Pheochromocytoma
- ☐ b. Renal artery stenosis
- ☐ c. Conns syndrome
- ☐ d. Coarctaion of aorta
- ☐ e. Caushing syndrome

The correct answer is: Coarctaion of aorta

Question 2

Not answered

Marked out of 1.00

60 years old female presented to ER department by episodic headache+ADs- tachycardia and sweating, her blood pressure was 170/95mmhg. Which is the main line of treatment?

Select one:

- ☐ a. ACE
- ☐ b. Surgical resection
- ☐ c. B blocker
- ☐ d. ARBs
- ☐ e. Alpha blocker

The correct answer is: Surgical resection

Question 3

Not answered

Marked out of 1.00

What is the first possibility for the cause of HTN?

Select one:

- ☐ a. Drug induced
- ☐ b. Intracranial lesion
- ☐ c. Essential HTN
- ☐ d. Thyroid abnormality
- ☐ e. Renal artery stenosis

The correct answer is: Renal artery stenosis

Question 4

Not answered

Marked out of 1.00

55 years old male diabetic patient presented by headache+ADs- vomiting, chest pain and blurring of vision+ADs- his blood pressure was 200/110mmhg. What is the first line of management?

Select one:

- ☐ a. IV fluids
- ☐ b. IV cannula
- ☐ c. ICU admission and IV antihypertensive
- ☐ d. Oral antihypertensive
- ☐ e. Oxygen therapy

The correct answer is: ICU admission and IV antihypertensive

Question 5

Not answered

Marked out of 1.00

All the following must be screened for secondary hypertension except:

Select one:

- ☐ a. Severe HTN
- ☐ b. Those with +IBw-essential hypertension+IB0- who remain hypertensive despite increasing dosages and numbers of antihypertensive medications, i.e., those refractory to what should normally be effective therapy.
- ☐ c. Those with a key feature of history, physical examination, or lab abnormality consistent with a particular form.
- ☐ d. Those who become hypertensive age +ADw-25 or +AD4-55
- ☐ e. Stage 1 HTN

The correct answer is: Stage 1 HTN

Question 6

Not answered

Marked out of 1.00

60 years old female presented to ER department by episodic headache+ADs- tachycardia and sweating, her blood pressure was 170/95mmhg. Which is the first laboratory investigation must be ordered for this case?

Select one:

- ☐ a. Urinary metanephrens .
- ☐ b. Serum aldosterone.
- ☐ c. Serum potassium(K).
- ☐ d. Serum renin.
- ☐ e. Serum sodium (Na).

The correct answer is: Urinary metanephrens .

Question 7

Not answered

Marked out of 1.00

55 years old male diabetic patient presented by headache+ADs- vomiting, chest pain and blurring of vision+ADs- his blood pressure was 200/110mmhg. All of the following investigations must be ordered urgently except?

Select one:

- ☐ a. CT brain
- ☐ b. ECG
- ☐ c. kidney function.
- ☐ d. Thyroid function.
- ☐ e. Fundus examination.

The correct answer is: Thyroid function.

Question 8

Not answered

Marked out of 1.00

60 years old female presented to ER department by episodic headache+ADs- tachycardia and sweating, her blood pressure was 170/95mmhg. What is the first possibility of the cause of HTN in this case?

Select one:

- ☐ a. Coarctation of aorta.
- ☐ b. Oral contraceptive pills
- ☐ c. Caushing syndrome
- ☐ d. Pheochromocytoma
- ☐ e. Renal artery stenosis

The correct answer is: Pheochromocytoma

Question 9

Not answered

Marked out of 1.00

25 years old female patient presented to emergency department by new onset blood pressure of 180/110 which is resistant to treatment. What is the gold standard investigation for diagnosis of cause of HTN in this case?

Select one:

- ☐ a. CT brain
- ☐ b. Echocardiography
- ☐ c. Duplex ultrasound
- ☐ d. Abdominal US
- ☐ e. Renal arteriography

The correct answer is: Renal arteriography

Question 10

Not answered

Marked out of 1.00

18 years old young man presented by hypertension which is 160/90 in upper limb and 120/75 in lower limbs. What is the best modality for diagnosis of the cause of hypertension?

Select one:

- ☐ a. CT abdomen
- ☐ b. CT brain
- ☐ c. abdominal US
- ☐ d. X ray chest
- ☐ e. CT aortography

The correct answer is: CT aortography

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Completed on Saturday, 27 May 2023, 3:04 PM
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Marks 0.00/4.00
Grade **0.00** out of 10.00 (0%)

Question 1

Not answered

Marked out of 1.00

As primary hyperlipidemia progresses, the following signs and symptoms develop, EXCEPT:

Select one:

- ☐ a. severe hypertriglyceridemia.
- ☐ b. lipemic plasma.
- ☐ c. obesity.
- ☐ d. eruptive xanthomas.

The correct answer is: obesity.

Question 2

Not answered

Marked out of 1.00

Increased LDL levels can result from

Select one:

- ☐ a. a deficiency of estrogens.
- ☐ b. genetic mutations of either the LDL receptor or Apo B-100.
- ☐ c. a deficiency of thyroid hormones.
- ☐ d. All of the above

The correct answer is: All of the above

Question 3

Not answered

Marked out of 1.00

Secondary hyperlipidemia with elevated cholesterol is the main dyslipidemia in patients with

Select one:

- ☐ a. hormone replacement therapy.
- ☐ b. alcohol abuse.
- ☐ c. obesity.
- ☐ d. chronic renal failure.

The correct answer is: chronic renal failure.

Question 4

Not answered

Marked out of 1.00

What is the main symptom of high cholesterol?

Select one:

- ☐ a. Weight gain
- ☐ b. Fatigue
- ☐ c. Non of above
- ☐ d. Weakness

The correct answer is: Non of above

Started on Friday, 2 June 2023, 6:08 PM
State Finished
Completed on Friday, 2 June 2023, 6:08 PM
Time taken 33 secs
Grade 0.00 out of 10.00 (0%)

Question 1

Not answered

Marked out of 1.00

25 year old patient comes to the internal medicine clinic complaining of 2 weeks of fatigue and cough. The patient admits that he is a heroin addict. On examination, pulse: 110 beat/minute and regular, BP: 110/80, T: 38.8 RR: 25 /minute. A holosystolic murmur is heard at the left lower sternal border. scattered crackles are heard in lung examination. Mild splenomegaly in abdomen examination. There is swelling and tenderness in the right and left ankle joint with some palpable purpuric eruptions on both legs. A normal neurological examination. What is the most likely diagnosis

Select one:

- ☐ a. Systemic lupus erythomatosus
- ☐ b. Serum thickness
- ☐ c. Infectious mononeuclosis
- ☐ d. Rheumatoid artheritis
- ☐ e. Acute rheumatic fever

The correct answer is: Acute rheumatic fever

Question 2

Not answered

Marked out of 1.00

Which of the following is an essential part of the diagnosis of acute rheumatic fever in this patient?

Select one:

- ☐ a. CNS affection
- ☐ b. Positive blood culture
- ☐ c. Positive streptococcal culture from joint aspiration
- ☐ d. High titer of rheumatoid factor
- ☐ e. Evidence of streptococcal infection

The correct answer is: Evidence of streptococcal infection

Question 3

Not answered

Marked out of 1.00

The holosystolic murmur in that patient signifies affection of which of the following?

Select one:

- ☐ a. Swelling of MV leaflets
- ☐ b. Myocarditis
- ☐ c. Pericardial effusion
- ☐ d. Destruction of MV leaflets
- ☐ e. Conduction system

The correct answer is: Destruction of MV leaflets

Question 4

Not answered

Marked out of 1.00

What is the neurological affection in acute rheumatic fever?

Select one:

- ☐ a. Tremors
- ☐ b. Hemiplegia
- ☐ c. Ataxia
- ☐ d. Chorea
- ☐ e. Hemisensory loss

The correct answer is: Chorea

Question 5

Not answered

Marked out of 1.00

What is the skin affection in acute rheumatic fever?

Select one:

- ☐ a. Erythema marginatum
- ☐ b. Necrolytic migratory erythema
- ☐ c. Erythema toxicum
- ☐ d. Erythema multiforme
- ☐ e. Erythema nodosum

The correct answer is: Erythema marginatum

Question 6

Not answered

Marked out of 1.00

17 year old patient comes to the internal medicine clinic complaining of knee and ankle pain associated with dyspnea and orthopnea. The patient mentions that he had a sore throat 2 weeks ago that went away with the aid of some herbal remedies. On examination, pulse: 110 beat/minute and regular BP: 110/80, T: 37.8 °C RR: 25 /minute. There is swelling and tenderness in the right and left ankle joint. Apex is hyperkinetic with 3rd heart sound and holosystolic murmur heard at the apex and radiate to the axilla. Bibasal crackles are heard in lung examination. The abdomen is soft without tenderness. A normal neurological examination. A throat swab is positive for streptococcus pyogenes. What could prevent this non-suppurative streptococcal sequel?

Select one:

- ☐ a. Prompt administration of penicillin in streptococcal sore throat
- ☐ b. Prompt administration of salicylic acid in streptococcal sore throat
- ☐ c. No effective way for primary prevention.
- ☐ d. Tonsillectomy
- ☐ e. Continues warm water throat washing.

The correct answer is: Prompt administration of penicillin in streptococcal sore throat

Question 7

Not answered

Marked out of 1.00

What is the treatment of this patient current condition?

Select one:

- ☐ a. Acetaminophen
- ☐ b. Aspirin
- ☐ c. Aspirin, steroid, penicillin and diuretic
- ☐ d. Aspirin and steroid
- ☐ e. Aspirin, steroid, and penicillin

The correct answer is: Aspirin, steroid, penicillin and diuretic

Question 8

Not answered

Marked out of 1.00

What is the expected sequel of arthritis in acute rheumatic fever?

Select one:

- ☐ a. Joint contracture
- ☐ b. Total resolution
- ☐ c. Persistent effusion
- ☐ d. Joint destruction
- ☐ e. Pannus formation

The correct answer is: Total resolution

Question 9

Not answered

Marked out of 1.00

What is the expected sequel of repeated carditis in acute rheumatic fever?

Select one:

- ☐ a. Chronic rheumatic heart disease
- ☐ b. Hypertrophic cardiomyopathy
- ☐ c. Ventricular septal defect
- ☐ d. Restrictive pericarditis
- ☐ e. Total resolution

The correct answer is: Chronic rheumatic heart disease

Question 10

Not answered

Marked out of 1.00

What is the best secondary prevention for this patient?

Select one:

- ☐ a. Ceftriaxone 1 gram every 15 days
- ☐ b. Rifampicin for 6 months
- ☐ c. Secondary prevention is not cost-effective as recurrence is unlikely.
- ☐ d. Benzathine penicillin G every 15-30 day deep intramuscular
- ☐ e. No effective secondary prevention

The correct answer is: Benzathine penicillin G every 15-30 day deep intramuscular

Started on Friday, 2 June 2023, 6:08 PM
State Finished
Completed on Friday, 2 June 2023, 6:09 PM
Time taken 39 secs
Marks 0.00/5.00
Grade **0.00** out of 10.00 (0%)

Question 1

Not answered

Marked out of 1.00

25 year old patient comes to the internal medicine clinic complaining of 2 weeks of fatigue and cough. The patient admits that he is a heroin addict. On examination, pulse: 110 beat/minute and regular+ADs- BP: 110/80, T: 38.8 cd+ADs- RR: 25 /minute. A holosystolic murmur is heard at the left lower sternal border. scattered crackles are heard in lung examination. Mild splenomegaly in abdomen examination. There is swelling and tenderness in the right and left ankle joint with some palpable purpuric eruptions on both legs. A normal neurological examination. What is the most likely cause of abnormal lung auscultation in this patient?

Select one:

- ☐ a. Sarcoidosis
- ☐ b. Interstitial pulmonary fibrosis
- ☐ c. Multiple septic pulmonary emboli
- ☐ d. Alveolar hemorrhage
- ☐ e. Malignancy

The correct answer is: Multiple septic pulmonary emboli

Question 2

Not answered

Marked out of 1.00

25 year old patient comes to the internal medicine clinic complaining of 2 weeks of fatigue and cough. The patient admits that he is a heroin addict. On examination, pulse: 110 beat/minute and regular+ADs- BP: 110/80, T: 38.8 cd+ADs- RR: 25 /minute. A holosystolic murmur is heard at the left lower sternal border. scattered crackles are heard in lung examination. Mild splenomegaly in abdomen examination. There is swelling and tenderness in the right and left ankle joint with some palpable purpuric eruptions on both legs. A normal neurological examination. What is the best next step in management?

Select one:

- ☐ a. Cardiac MRI
- ☐ b. Start empiric antibiotic.
- ☐ c. Multiple sets of blood culture
- ☐ d. Cardiac CT
- ☐ e. Echocardiography

The correct answer is: Multiple sets of blood culture

Question 3

Not answered

Marked out of 1.00

25 year old patient comes to the internal medicine clinic complaining of 2 weeks of fatigue and cough. The patient admits that he is a heroin addict. On examination, pulse: 110 beat/minute and regular+ADs- BP: 110/80, T: 38.8 cd+ADs- RR: 25 /minute. A holosystolic murmur is heard at the left lower sternal border. scattered crackles are heard in lung examination. Mild splenomegaly in abdomen examination. There is swelling and tenderness in the right and left ankle joint with some palpable purpuric eruptions on both legs. A normal neurological examination. What is the most likely organism causing that disease?

Select one:

- ☐ a. E. coli
- ☐ b. Staph. Aureus
- ☐ c. Stript. Bovis.
- ☐ d. Stript. Viridans
- ☐ e. Coxiella burnetiid

The correct answer is: Staph. Aureus

Question 4

Not answered

Marked out of 1.00

25 year old patient comes to the internal medicine clinic complaining of 2 weeks of fatigue and cough. The patient admits that he is a heroin addict. On examination, pulse: 110 beat/minute and regular+ADs- BP: 110/80, T: 38.8 cd+ADs- RR: 25 /minute. A holosystolic murmur is heard at the left lower sternal border. scattered crackles are heard in lung examination. Mild splenomegaly in abdomen examination. There is swelling and tenderness in the right and left ankle joint with some palpable purpuric eruptions on both legs. A normal neurological examination. What does palpable purpura indicate in this patient?

Select one:

- ☐ a. Immune complex mediate vasculitis
- ☐ b. Viral exanthem
- ☐ c. Hereditary hemorrhagic telangiectasia
- ☐ d. Staph. Aureus embolization
- ☐ e. Thrombocytopenia of sepsis

The correct answer is: Immune complex mediate vasculitis

Question 5

Not answered

Marked out of 1.00

25 year old patient comes to the internal medicine clinic complaining of 2 weeks of fatigue and cough. The patient admits that he is a heroin addict. On examination, pulse: 110 beat/minute and regular+ADs- BP: 110/80, T: 38.8 cd+ADs- RR: 25 /minute. A holosystolic murmur is heard at the left lower sternal border. scattered crackles are heard in lung examination. Mild splenomegaly in abdomen examination. There is swelling and tenderness in the right and left ankle joint with some palpable purpuric eruptions on both legs. A normal neurological examination. What is the cardiac valve most likely affected in this patient?

Select one:

- ☐ a. TV
- ☐ b. MV
- ☐ c. AV
- ☐ d. PV.

The correct answer is: TV

Started on Friday, 2 June 2023, 6:08 PM
State Finished
Completed on Friday, 2 June 2023, 6:09 PM
Time taken 41 secs
Marks 0.00/8.00
Grade **0.00** out of 10.00 (0%)

Question 1

Not answered

Marked out of 1.00

A patient with Ehler-Danols syndrome complained from chest pain and suddenly arrested, post mortum pathological examination revealed that cause of death was:

Select one:

- ☐ a. Cerebral hemorrhage.
- ☐ b. Acute coronary syndrome.
- ☐ c. Aortic dissection.
- ☐ d. None of above.

The correct answer is: Aortic dissection.

Question 2

Not answered

Marked out of 1.00

In Down syndrome the following are common cardiac association:

Select one:

- ☐ a. ASD.
- ☐ b. Mitral regurge.
- ☐ c. Aortic regurge.
- ☐ d. ASD and Mitral regurge.

The correct answer is: ASD and Mitral regurge.

Question 3

Not answered

Marked out of 1.00

Aortic regurge is detected in the echocardiography of the following conditions:

Select one:

- ☐ a. None of above.
- ☐ b. Hyperthyroidism.
- ☐ c. Rheumatoid arthritis.
- ☐ d. Hypothyroidism.

The correct answer is: Rheumatoid arthritis.

Question 4

Not answered

Marked out of 1.00

Pericardial effusion can be detected in the following diseases:

Select one:

- ☐ a. Systemic lupus erytromatosis and Rheumatoid arthritis.
- ☐ b. Cushing syndrome.
- ☐ c. Rheumatoid arthritis.
- ☐ d. Systemic lupus erytromatosis.

The correct answer is: Systemic lupus erytromatosis and Rheumatoid arthritis.

Question 5

Not answered

Marked out of 1.00

A case of secondary hypertension was suspected, his laboratory data revealed high insulin-like growth factor and high levels of growth hormone, on examination the patient is:

Select one:

- ☐ a. Muddy face, cachectic and on regular dialysis.
- ☐ b. Tall, large hands and feet, prominent forehead, and prognathism.
- ☐ c. None of above.
- ☐ d. Short, obese and with webbed neck.

The correct answer is: Tall, large hands and feet, prominent forehead, and prognathism.

Question 6

Not answered

Marked out of 1.00

A 37 years old male complaining from low back pain and sacroiliitis, ECG showed AV block, the most possible diagnosis is:

Select one:

- ☐ a. Systemic lupus erythromatosis.
- ☐ b. Rheumatoid arthritis.
- ☐ c. Carcinoid syndrome.
- ☐ d. Ankylosing spondylitis.

The correct answer is: Ankylosing spondylitis.

Question 7

Not answered

Marked out of 1.00

A 29 year old female, had thyroid swelling and diagnosed as hyperthyroidism, on examination her BP is expected to be:

Select one:

- ☐ a. Narrow pulse pressure.
- ☐ b. Wide pulse pressure.
- ☐ c. low BP.
- ☐ d. High BP.

The correct answer is: Wide pulse pressure.

Question 8

Not answered

Marked out of 1.00

Atrial fibrillation is a common complication in the following systemic disorders.:

Select one:

- ☐ a. Hypothyroidism.
- ☐ b. Hemochromatosis.
- ☐ c. None of above.
- ☐ d. Hemochromatosis and Hypothyroidism.

The correct answer is: Hemochromatosis.

Started on Saturday, 10 June 2023, 2:54 PM
State Finished
Completed on Saturday, 10 June 2023, 3:16 PM
Time taken 21 mins 32 secs
Marks 0.00/5.00
Grade 0.00 out of 10.00 (0%)

Question 1

Not answered

Marked out of 1.00

A 62-year-old woman with type 2 diabetes mellitus comes to the physician because of a 3-month history of fatigue and weakness. Her blood pressure is 150/90mmHg. Examination shows lower limb edema.

K ⁺	5.1
phosphorus	5 mg/dl
Ca	7.8 mg /dl
Urea nitrogen	60 mg/dl
Creatinine	2.2 mg/dl

Which of the following is the most appropriate parameter for early detection of this patient's renal condition?

Select one:

- ☐ a. Serum total protein
- ☐ b. Urinary red blood cell cast
- ☐ c. Serum creatinine
- ☐ d. Urinary albumin

Your answer is incorrect.

The correct answer is: Urinary albumin

Question 2

Not answered

Marked out of 1.00

A 43-year-old man with 1-week history of swelling around his eyes and decreased urination. His blood pressure is 155/95 mmHg. His physical examination shows periorbital and lower limb edema. Serum urea nitrogen concentration 21 mg/dl and creatinine is 1.4 mg/dl.

Urinalysis shows 3+ blood and 1+ protein.

Further evaluation of this patient is most likely to show which of the following?

Select one:

- ☐ a. Detached renal tubular epithelial cells
- ☐ b. Hypoalbuminemia
- ☐ c. Red blood cell cast
- ☐ d. Urinary rhomboid crystals

Your answer is incorrect.

The correct answer is: Red blood cell cast

Question 3

Not answered

Marked out of 1.00

A 20-year-old man comes to the physician because of dark urine and decreased urine output for 2 days. He had a skin infection that required antibiotic treatment 3 weeks ago. His blood pressure is 140/90 mmHg. He has periorbital edema.

What is the most likely diagnosis?

Select one:

- ☐ a. IgA nephropathy
- ☐ b. Poststreptococcal glomerulonephritis
- ☐ c. Minimal change disease
- ☐ d. Membranous GN

Your answer is incorrect.

The correct answer is: Poststreptococcal glomerulonephritis

Question 4

Not answered

Marked out of 1.00

A 20-year-old athletic man with no significant past medical history is found to have isolated asymptomatic microscopic hematuria on routine exam. Which of the following suggests hematuria of glomerular origin?

Select one:

- ☐ a. Presence of dysmorphic RBCS in urine sediment
- ☐ b. Presence of isomorphic WBCS in urine sediment
- ☐ c. Presence of decoy cells in urine sediment
- ☐ d. Presence of isomorphic RBCS in urine sediment

Your answer is incorrect.

The correct answer is: Presence of dysmorphic RBCS in urine sediment

Question 5

Not answered

Marked out of 1.00

Which of the following malignancies associated with minimal change disease (MCD)?

Select one:

- ☐ a. Lung cancer
- ☐ b. Colon and breast cancer
- ☐ c. Chronic lymphocytic leukemia
- ☐ d. Multiple myeloma

Your answer is incorrect.

The correct answer is: Chronic lymphocytic leukemia

Started on Friday, 16 June 2023, 4:48 PM
State Finished
Completed on Friday, 16 June 2023, 4:48 PM
Time taken 11 secs
Marks 0.00/6.00
Grade **0.00** out of 10.00 (0%)

Question 1

Not answered

Marked out of 1.00

Acute Kidney Injury (AKI) is defined by rapid deterioration of GFR (within two days) that persist for

Select one:

- ☐ a. Less than 3 months
- ☐ b. More than 6 months
- ☐ c. More than 3 months
- ☐ d. Less than 5 months

Your answer is incorrect.

The correct answer is: Less than 3 months

Question 2

Not answered

Marked out of 1.00

The most common type of AKI is:

Select one:

- ☐ a. Glomerulonephritis (GN).
- ☐ b. Interstitial nephritis
- ☐ c. Prerenal –AKI.
- ☐ d. Postrenal –AKI

Your answer is incorrect.

The correct answer is: Prerenal –AKI.

Question 3

Not answered

Marked out of 1.00

As regard Acute Tubula Necrosis (ATN), the urine analysis show:

Select one:

- ☐ a. Pigmented broad casts.
- ☐ b. Bland urine sediments
- ☐ c. Eosinophil casts
- ☐ d. RBCs casts

Your answer is incorrect.

The correct answer is: Pigmented broad casts.

Question 4

Not answered

Marked out of 1.00

We refer the AKI patients to dialysis in cases of

Select one:

- ☐ a. Hypokalaemia
- ☐ b. Hypernatremia
- ☐ c. Metabolic acidosis
- ☐ d. Metabolic alkalosis

Your answer is incorrect.

The correct answer is: Metabolic acidosis

Question 5

Not answered

Marked out of 1.00

Rhabdomyolysis can lead totype of AKI:

Select one:

- ☐ a. Acute Tubular Necrosis (ATN).
- ☐ b. Interstitial nephritis.
- ☐ c. Postrenal
- ☐ d. Glomerulonephritis (GN).

Your answer is incorrect.

The correct answer is: Acute Tubular Necrosis (ATN).

Question 6

Not answered

Marked out of 1.00

Which of the following is associated with good prognosis in AKI

Select one:

- ☐ a. Poluric AKI
- ☐ b. Post traumatic AKI
- ☐ c. Oliguric AKI
- ☐ d. Septic AKI

Your answer is incorrect.

The correct answer is: Poluric AKI

Started on Friday, 23 June 2023, 10:42 PM

State Finished

Completed on Friday, 23 June 2023, 10:43 PM

Time taken 50 secs

Marks 0.00/18.00

Grade 0.00 out of 10.00 (0%)

Question 1

Not answered

Marked out of 1.00

Chronic Kidney Disease (CKD) is defined AS

Select one:

- ☐ a. GFR < 60 ml/min/1.73 m2 for less than 3 months
- ☐ b. Kidney damage as proteinuria for 2 months
- ☐ c. Glomerular Filtration Rate (GFR) < 50 ml/min/1.73 m2 for at least 3 months
- ☐ d. GFR < 60 ml/min/1.73 m2 with or without kidney damage for at least 3 months

Your answer is incorrect.

The correct answer is: GFR < 60 ml/min/1.73 m2 with or without kidney damage for at least 3 months

Question 2

Not answered

Marked out of 1.00

CKD is a common disease worldwide with:

Select one:

- ☐ a. More common in males than females
- ☐ b. No sex difference
- ☐ c. Increased incidence in middle age groups 40-60 yrs
- ☐ d. Increased incidence in older age > 65 yrs and female sex

Your answer is incorrect.

The correct answer is: Increased incidence in older age > 65 yrs and female sex

Question 3

Not answered

Marked out of 1.00

The most common cause of CKD in Egypt:

Select one:

- ☐ a. Stones of the urinary tract
- ☐ b. Diabetes & Hypertension
- ☐ c. ADPKD
- ☐ d. Chronic Glomerulonephritis & drugs addiction

Your answer is incorrect.

The correct answer is: Diabetes & Hypertension

Question 4

Not answered

Marked out of 1.00

As regard anaemia of CKD

Select one:

- ☐ a. Macrocytic anaemia is the most common type
- ☐ b. Iron deficiency (IDA) is the most common type
- ☐ c. All types of anaemia can occur
- ☐ d. Pernicious anaemia is more common than Normocytic Normochromic anaemia

Your answer is incorrect.

The correct answer is: All types of anaemia can occur

Question 5

Not answered

Marked out of 1.00

The most common cause of anaemia of CKD is

Select one:

- ☐ a. Folate deficiency
- ☐ b. Erthropoietin hormone deficiency
- ☐ c. Copper deficiency
- ☐ d. Iron deficiency

Your answer is incorrect.

The correct answer is: Erthropoietin hormone deficiency

Question 6

Not answered

Marked out of 1.00

In CKD-MBD(mineral bone disease):-

Select one:

- ☐ a. Osteopenia is adynamic bone disease
- ☐ b. Osteomalacia is a high turn over bone disease
- ☐ c. Osteoporosis is a low turn over bone disease
- ☐ d. Osteitis Fibrosa Cystica is a low turn over bone disease

Your answer is incorrect.

The correct answer is: Osteoporosis is a low turn over bone disease

Question 7

Not answered

Marked out of 1.00

Acute Kidney Injury (AKI) is defined by rapid deterioration of GFR (within two days) that persist for

Select one:

- ☐ a. Less than 5 months
- ☐ b. More than 6 months
- ☐ c. More than 3 months
- ☐ d. Less than 3 months

Your answer is incorrect.

The correct answer is: Less than 3 months

Question 8

Not answered

Marked out of 1.00

The most common type of AKI is

Select one:

- ☐ a. Interstitial nephritis
- ☐ b. Prerenal –AKI
- ☐ c. Glomerulonephritis (GN)
- ☐ d. Postrenal –AKI.

Your answer is incorrect.

The correct answer is: Prerenal –AKI

Question 9

Not answered

Marked out of 1.00

As regard Acute Tubula Necrosis (ATN), the urine analysis show

Select one:

- ☐ a. Bland urine sediments
- ☐ b. Pigmented broad casts
- ☐ c. RBCs casts
- ☐ d. Eosinophil casts

Your answer is incorrect.

The correct answer is: Pigmented broad casts

Question 10

Not answered

Marked out of 1.00

We refer the AKI patients to dialysis in cases of

Select one:

- ☐ a. Hypermnatremia
- ☐ b. Metabolic acidosis
- ☐ c. Metabolic alkalosis
- ☐ d. Hypokalaemia

Your answer is incorrect.

The correct answer is: Metabolic acidosis

Question 11

Not answered

Marked out of 1.00

Rhabdomyolysis can lead totype of AKI

Select one:

- ☐ a. Postrenal
- ☐ b. Glomerulonephritis (GN)
- ☐ c. Interstitial nephritis
- ☐ d. Acute Tubular Necrosis (ATN)

Your answer is incorrect.

The correct answer is: Acute Tubular Necrosis (ATN)

Question 12

Not answered

Marked out of 1.00

Which of the following is associated with good prognosis in AKI

Select one:

- ☐ a. Septic AKI
- ☐ b. Poluric AKI
- ☐ c. Post traumatic AKI
- ☐ d. Oliguric AKI

Your answer is incorrect.

The correct answer is: Poluric AKI

Question 13

Not answered

Marked out of 1.00

As regard Autosomal Recessive Polycystic Kidney(ARPKD):

Select one:

- ☐ a. It lead to renal failure in elderly > 65 yrs
- ☐ b. It lead to fetal deaths in intrauterine period & neonatal period
- ☐ c. It can lead to renal failure in young adult 20-40 yrs of age
- ☐ d. It is more common than ADPKD

Your answer is incorrect.

The correct answer is: It lead to fetal deaths in intrauterine period & neonatal period

Question 14

Not answered

Marked out of 1.00

The most common presentation of ADPKD is

Select one:

- ☐ a. ESRD (renal failure)
- ☐ b. Renal colic
- ☐ c. Stones formation
- ☐ d. Hypertension

Your answer is incorrect.

The correct answer is: Hypertension

Question 15

Not answered

Marked out of 1.00

The most common valve affected in ADPKD

Select one:

- ☐ a. Mitral valve
- ☐ b. Pulmonary valve
- ☐ c. Aortic valve
- ☐ d. Tricusid valve
- ☐ e. Pulmonary valve

Your answer is incorrect.

The correct answer is: Mitral valve

Question 16

Not answered

Marked out of 1.00

The most dangerous presentation of ADPKD IS:

Select one:

- ☐ a. ESRD (rena failure)
- ☐ b. Subarachinoid haemorrhage
- ☐ c. Hypertension
- ☐ d. Anaemia

Your answer is incorrect.

The correct answer is: Subarachinoid haemorrhage

Question 17

Not answered

Marked out of 1.00

Tolvaptan is a novel therapy for ADPKD, it is

Select one:

- ☐ a. **ADH –antagonist**
- ☐ b. **Aquaretic**
- ☐ c. **Natriuretic**
- ☐ d. **Causing hyponatremia**

Your answer is incorrect.

The correct answer is: **Aquaretic**

Question 18

Not answered

Marked out of 1.00

Screening for ADPKD in the family is indicated in cases of :

Select one:

- ☐ a. Elderly ADPKD > 65 yrs
- ☐ b. For ADPKD patients with positive family history of cerebral haemorrhage
- ☐ c. Risky jobs as tunnel workers
- ☐ d. All adult patients with ADPKD & ESRD(renal failure)

Your answer is incorrect.

The correct answer is: For ADPKD patients with positive family history of cerebral haemorrhage